FUNCTIONAL IMPROVEMENT

MOVING BEYOND DYSTROPHIN IN DMD



Dyne® THERAPEUTICS

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Disclosures

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- Compensation for ad hoc advisory boards/consulting activity with Biogen, Novartis, Sarepta Therapeutics, PTC Therapeutics, Scholar Rock, Pfizer, Catalyst, Lupin, Santhera, Entrada Therapeutics, Avidity, Solid, Regenxbio, Insmed, Keros, Precisionbio, Gruenenthal, Mesoblast, Dyne Therapeutics, and Italfarmaco
- Associate Editor – Muscle and Nerve

Susan Apkon

- Site PI for Dyne Therapeutics, Capricor, Sarepta Therapeutics, Satellos, Biohaven, ScholarRock, Biogen

DELIVER trial data: Functional outcomes in focus

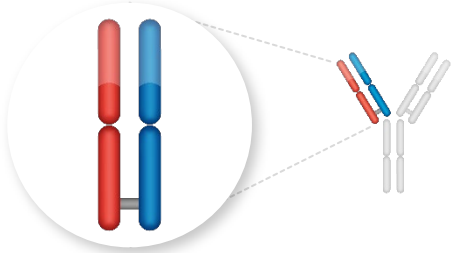
Susan Apkon, MD

Dyne THERAPEUTICS

Zeleciment rostudirsen (z-rostudirsen or DYNE-251) leverages the FORCE™ platform for broad delivery of an exon 51-skipping PMO to tissues impacted by DMD

ANTIBODY

Proprietary Fab against TfR1 to enable targeted delivery to muscle and CNS

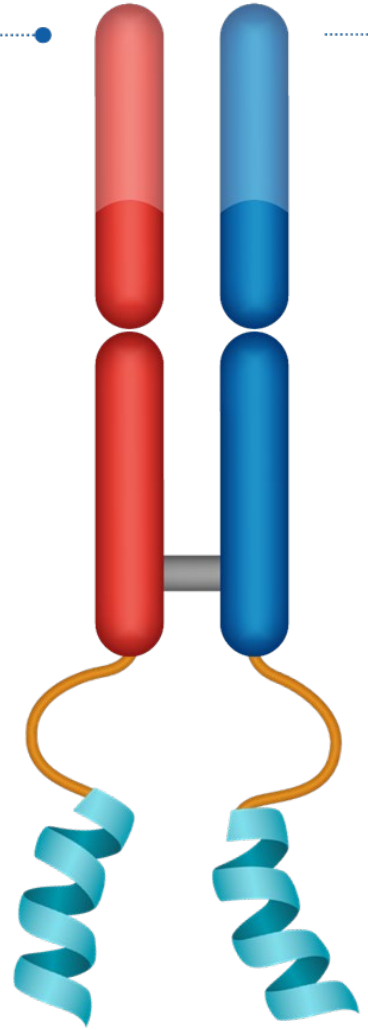


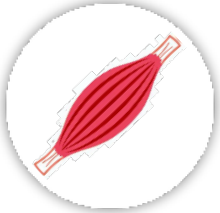
LINKER

Connects Fab and payload

PAYLOAD

PMO designed to enable production of near full-length dystrophin





Skeletal (including diaphragm) and smooth muscle



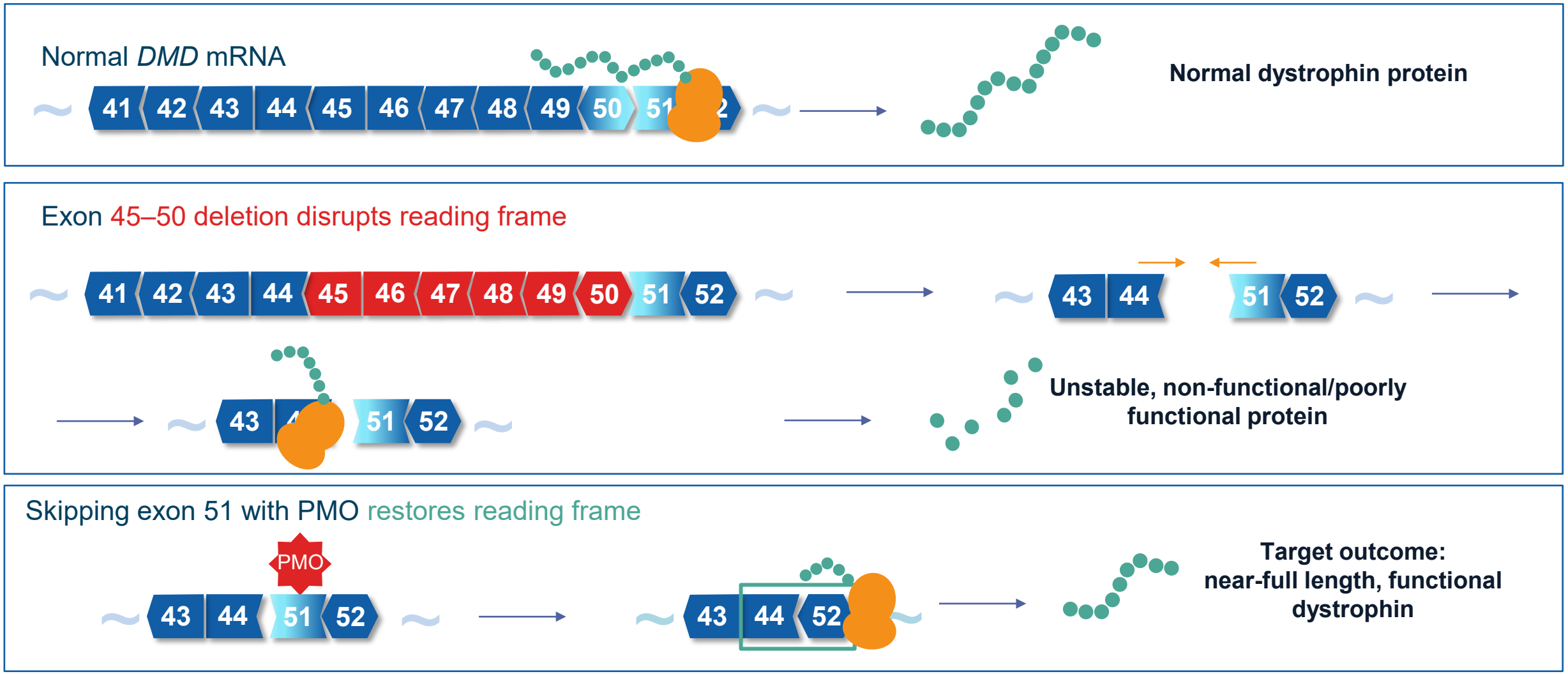
Cardiac muscle



CNS

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Note: Figure depicts oligonucleotide payload. CNS, central nervous system; DMD, Duchenne muscular dystrophy; Fab, fragment antibody-binding; PMO, phosphorodiamidate morpholino oligomer; TfR1, transferrin receptor 1.

PMO-induced exon skipping restores the *DMD* mRNA reading frame resulting in the production of near-full length, functional dystrophin

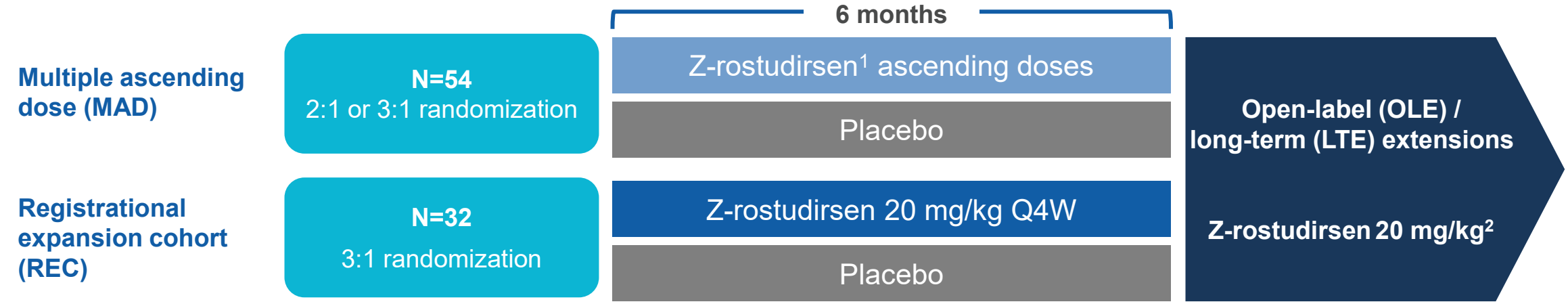


DMD, Duchenne muscular dystrophy; mRNA, messenger ribonucleic acid; PMO, phosphorodiamidate morpholino oligomer.

Niks EH, Aartsma-Rus A. *Expert Opin Biol Ther.* 2017;17:225–236.

DELIVER study design

DELIVER (N=86)



Select inclusion/exclusion criteria

Inclusion criteria

- Ambulatory or non-ambulatory
- Age 4 to 16 years inclusive
- Stable dosage of glucocorticoids for at least 12 weeks

Exclusion criteria

- Exon-skipping/dystrophin-modifying therapy or givinostat within 12 weeks of randomization
- Gene therapy at any time

Endpoints

Primary endpoints

- Change from baseline in dystrophin protein levels by Western Blot
- Safety and tolerability

Key functional endpoints

- TTR velocity, 10MWR velocity, NSAA, SV95C, PUL2.0, FVC%p

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1. Z-rostudirsen doses in the MAD cohorts ranged from 0.7 mg/kg to 40 mg/kg every 4 or 8 weeks. 2. Transition to 20 mg/kg dose occurred at non-uniform times during OLE or LTE; for participants initiated at 40 mg/kg, transition started either in the placebo-controlled period or OLE.

10MWR, 10-meter walk/run; FVC%p, forced vital capacity percent predicted; NSAA, North Star Ambulatory Assessment; PUL2.0, performance upper limb v2.0; Q4W, every 4 weeks; SV95C, stride velocity 95th centile; TTR, time to rise.

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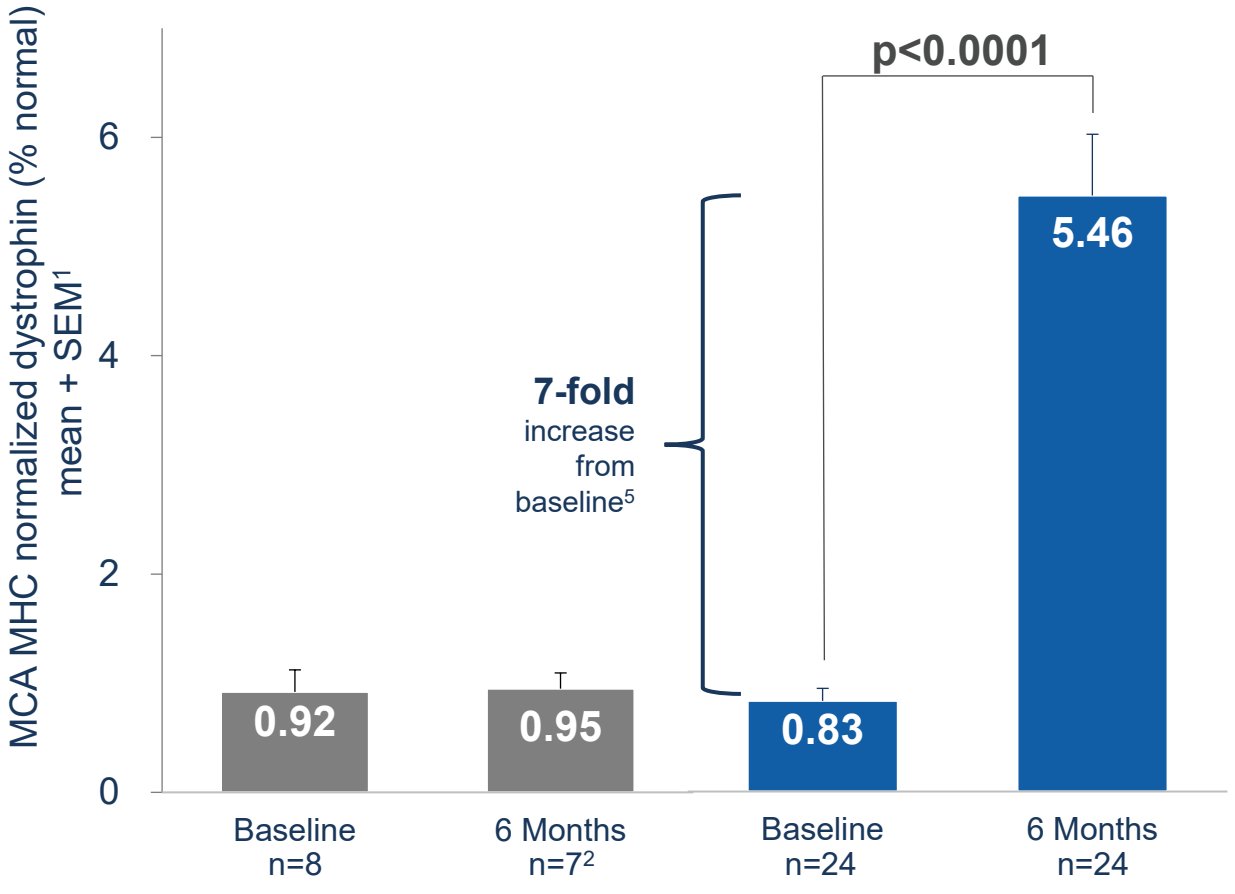
DELIVER baseline participant characteristics were generally well balanced across cohorts

	Placebo (MAD+REC) N=24[5] Mean (SD) or n (%)	Z-rostudirsen			
		20 mg/kg Q4W (REC) N=24 Mean (SD) or n (%)	10 → 20 mg/kg[6] Q4W (MAD) N=6 Mean (SD) or n (%)	20 mg/kg Q4W (MAD) N=6 Mean (SD) or n (%)	Overall (MAD+REC) N=86 Mean (SD) or n (%)
Age (years)	8.2 (2.5)	7.8 (3.6)	6.8 (2.5)	7.7 (2.5)	8.3 (3.3)
BMI (kg/m^2)	19.8 (4.7)	17.6 (4.5)	17.9 (3.7)	17.5 (2.9)	18.7 (4.3)
Age of symptom onset (years)	3.4 (1.8)	2.5 (1.7)	3.0 (1.8)	2.0 (0.9)	2.8 (1.7)
Most recent corticosteroid dosing regimen, n (%)[1] Daily Other	 20 (83.3) 4 (16.7)	 20 (83.3) 4 (16.7)	 6 (100) 0	 6 (100) 0	 73 (84.9) 13 (15.1)
Duration of corticosteroid treatment (years)[2]	2.1 (2.4)	2.4 (2.5)	1.5 (2.0)	2.4 (2.2)	2.6 (2.6)
Prior DMD therapy, n (%) Eteplirsen Other	 4 (16.7) 2 (8.3)	 2 (8.3) 5 (20.8)	 0 0	 0 2 (33.3)	 15 (17.4) 15 (17.4)
PUL2.0 total score[3]	36.3 (4.0)	36.3 (5.0)	37.2 (5.9)	33.8 (3.5)	36.4 (4.2)
FVC%p	92.7 (17.6)	90.0 (22.2)	89.8 (22.7)	90.7 (11.2)	87.5 (19.2)
Ambulant (%)	19 (79.2)	21 (87.5)	5 (83.3)	6 (100)	75 (87.2)
TTR velocity (rise/sec)[4]	0.20 (0.10)	0.22 (0.12)	0.23 (0.06)	0.17 (0.14)	0.20 (0.1)
10MWR velocity (m/sec)[4]	2.0 (0.5)	1.8 (0.5)	2.5 (0.7)	1.5 (0.6)	1.9 (0.6)
NSAA total score[4]	21.6 (6.3)	20.6 (5.0)	26.6 (5.4)	15.0 (5.3)	20.6 (7.0)
SV95C (m/sec)[4]	1.7 (0.5)	1.5 (0.4)	2.0 (0.3)	1.4 (0.5)	1.6 (0.5)

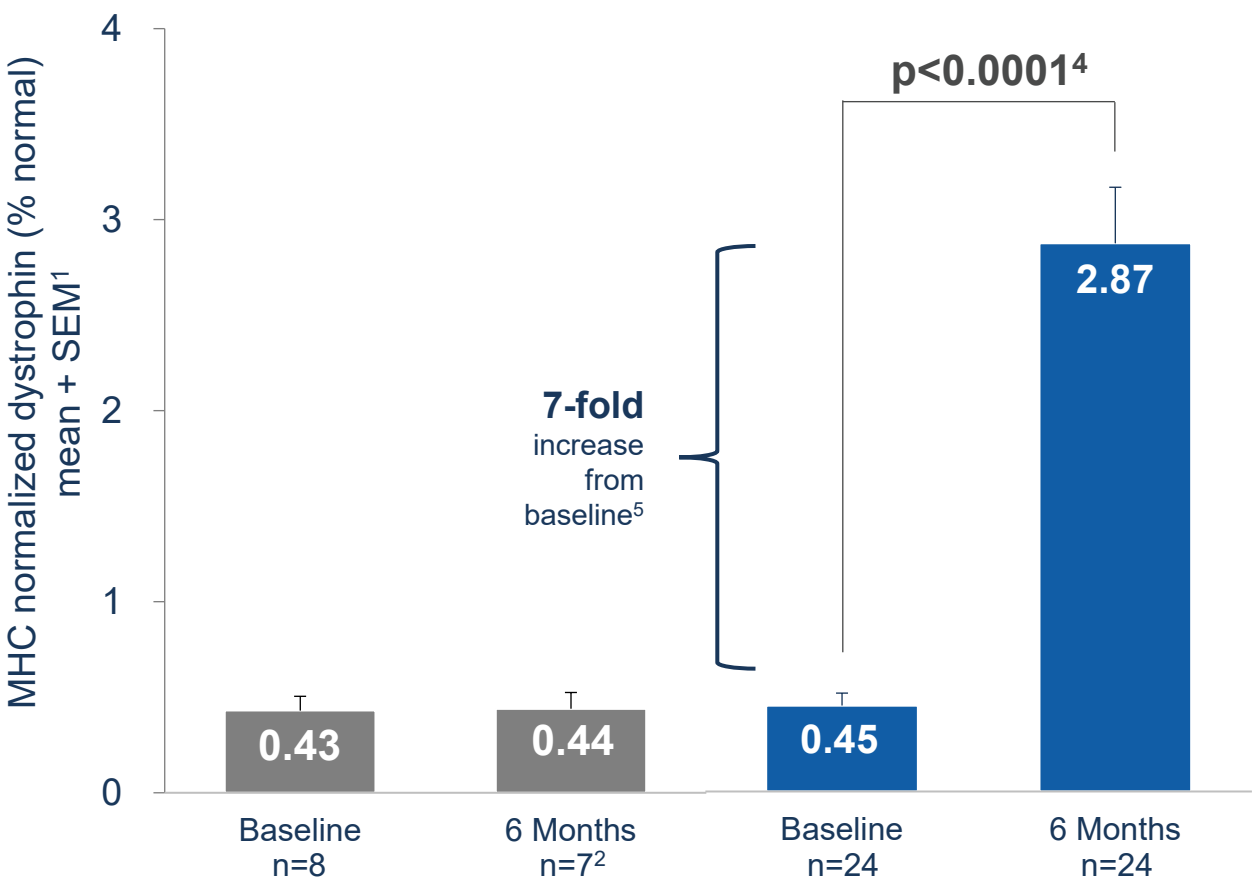
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1. Most recent corticosteroid regimen refers to corticosteroid at time of randomization. 2. Cumulative duration of previous and most recent corticosteroid treatment at the time of randomization. 3. Missing values imputed.
4. Ambulant participants; out-of-threshold and/or missing values imputed. 5. All placebo participants pooled from MAD and REC. 6. Participants transitioned from 10 mg/kg Q4W to 20 mg/kg Q4W after 6 months.
10MWR, 10-meter walk/run; BMI, body mass index; DMD, Duchenne muscular dystrophy; FVC%p, forced vital capacity percent predicted; kg, kilogram; m, meter; MAD, multiple ascending dose; NSAA, north star ambulatory assessment; PUL2.0, performance upper limb v2.0; Q4W, every 4 weeks; REC, registrational expansion cohort; SD, standard deviation; SV95C, stride velocity 95th centile; TTR, time to rise.
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Dystrophin expression with z-rostudirsen relative to baseline at 6 months

Muscle content-adjusted dystrophin[3] (REC)



Unadjusted dystrophin (REC)

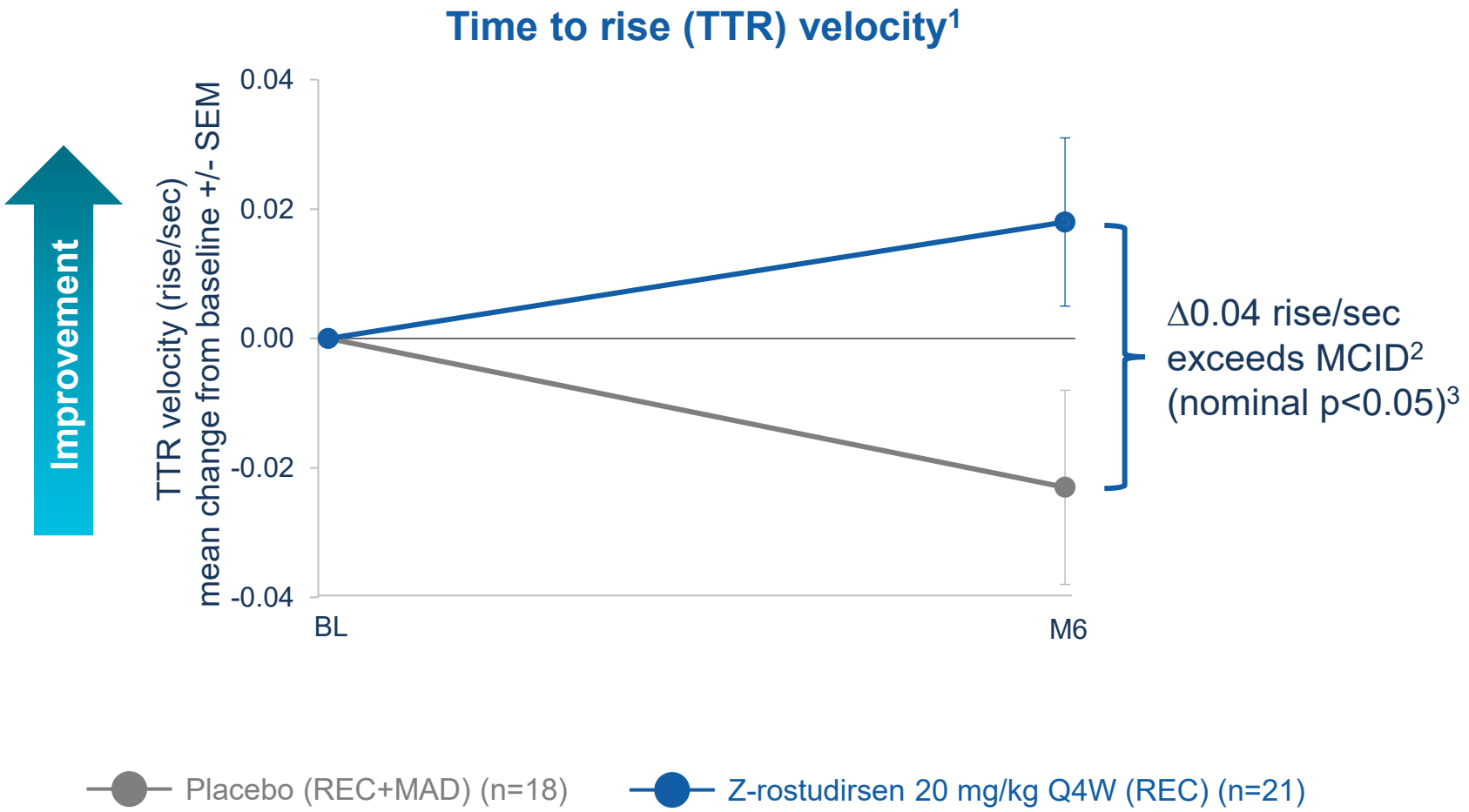


Placebo Z-rostudirsen 20 mg/kg Q4W (REC)

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1. Biopsies taken approximately 28 days after most recent dose. 2. One REC placebo participant sample could not be analyzed at Week 25. 3. Muscle content-adjusted dystrophin = MHC normalized dystrophin / % muscle content.
4. Prespecified nominal p-value with no adjustment for multiplicity. 5. Based on geometric mean.
6 months = Week 25 for DELIVER. MCA, muscle content-adjusted; MHC, myosin heavy chain; Q4W, every 4 weeks; REC, registrational expansion cohort; SEM, standard error of the mean.
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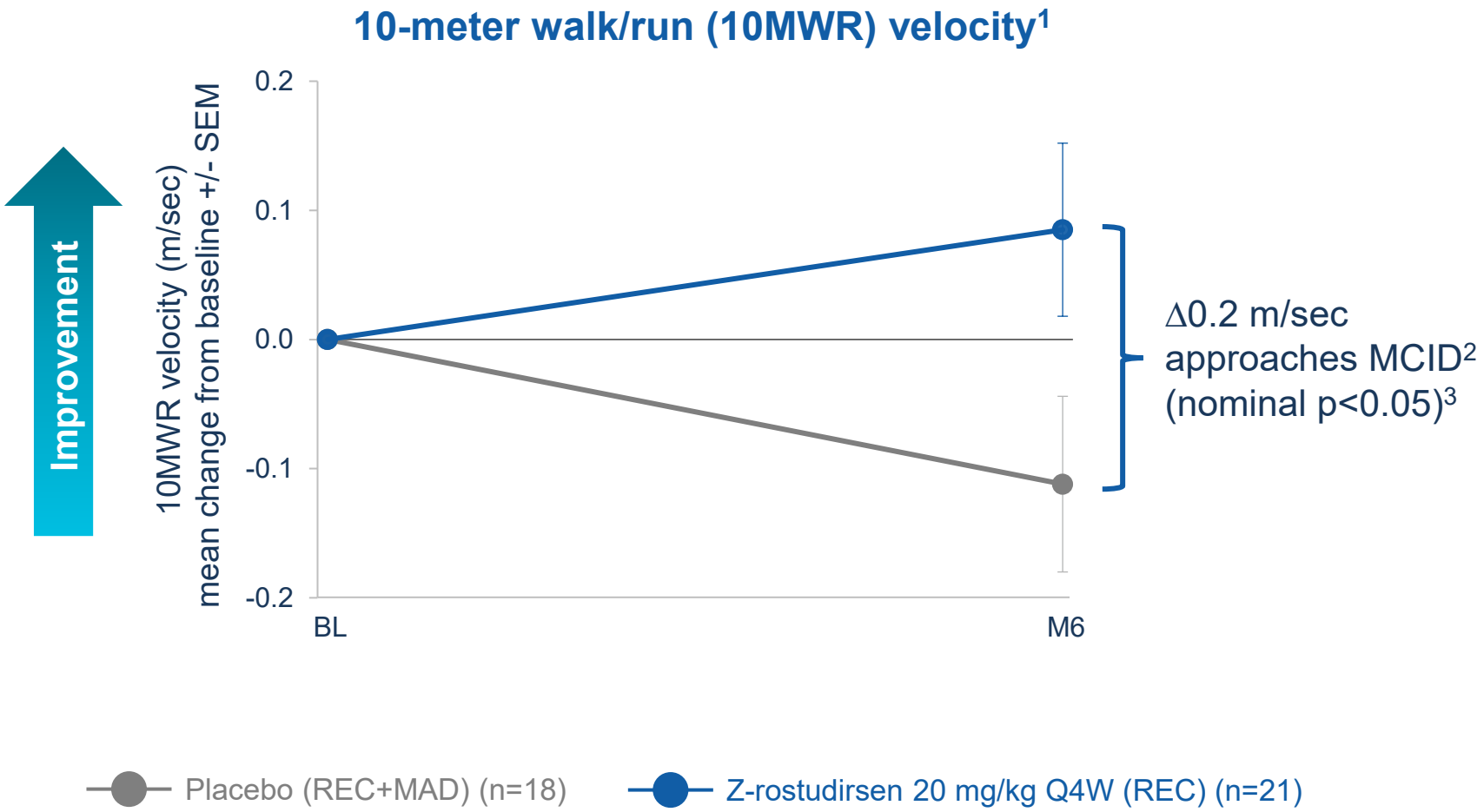
Increase in TTR velocity relative to placebo



Zelecitment rostudirsen (z-rostudirsen, also known as DYNE-251) is an investigational medicine or otherwise in development and has not been approved as safe or effective by the US FDA, EMA, or any other regulatory authority.

1. Ambulant participants; out-of-threshold or missing values imputed. 2. Duong T, et al. *J Neuromusc Dis.* 2021;8:939–948; MCID = 0.023 rise/sec. 3. Post-hoc analysis; prespecified statistical analysis plan did not include formal hypothesis testing for any functional endpoint.
BL, baseline; M, month; M6 = 169 days; MAD, multiple ascending dose; MCID, minimal clinically important difference; Q4W, every 4 weeks; REC, registrational expansion cohort; sec, second; SEM, standard error of the mean.
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Increase in 10MWR velocity relative to placebo



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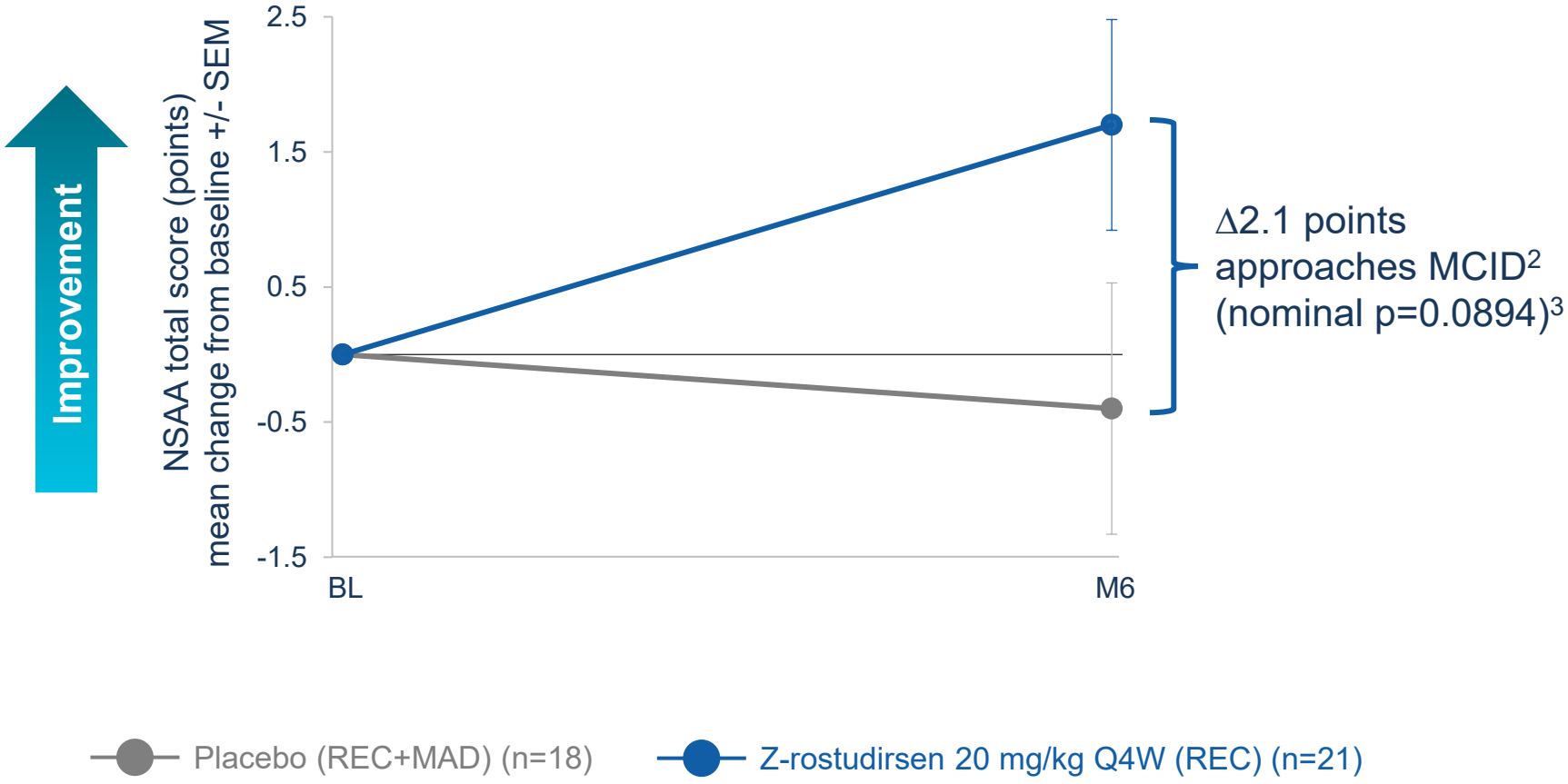
1. Ambulant participants; out-of-threshold or missing values imputed. 2. Duong T, et al. *J Neuromusc Dis.* 2021;8:939–948; MCID = 0.212 m/sec. 3. Post-hoc analysis; prespecified statistical analysis plan did not include formal hypothesis testing for any functional endpoint.

BL, baseline; M, month; m, meters; M6 = 169 days; MAD, multiple ascending dose; MCID, minimal clinically important difference; Q4W, every 4 weeks; REC, registrational expansion cohort; sec, second; SEM, standard error of the mean.

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Increase in NSAA at 6 months relative to baseline and placebo

North Star Ambulatory Assessment (NSAA)[1]



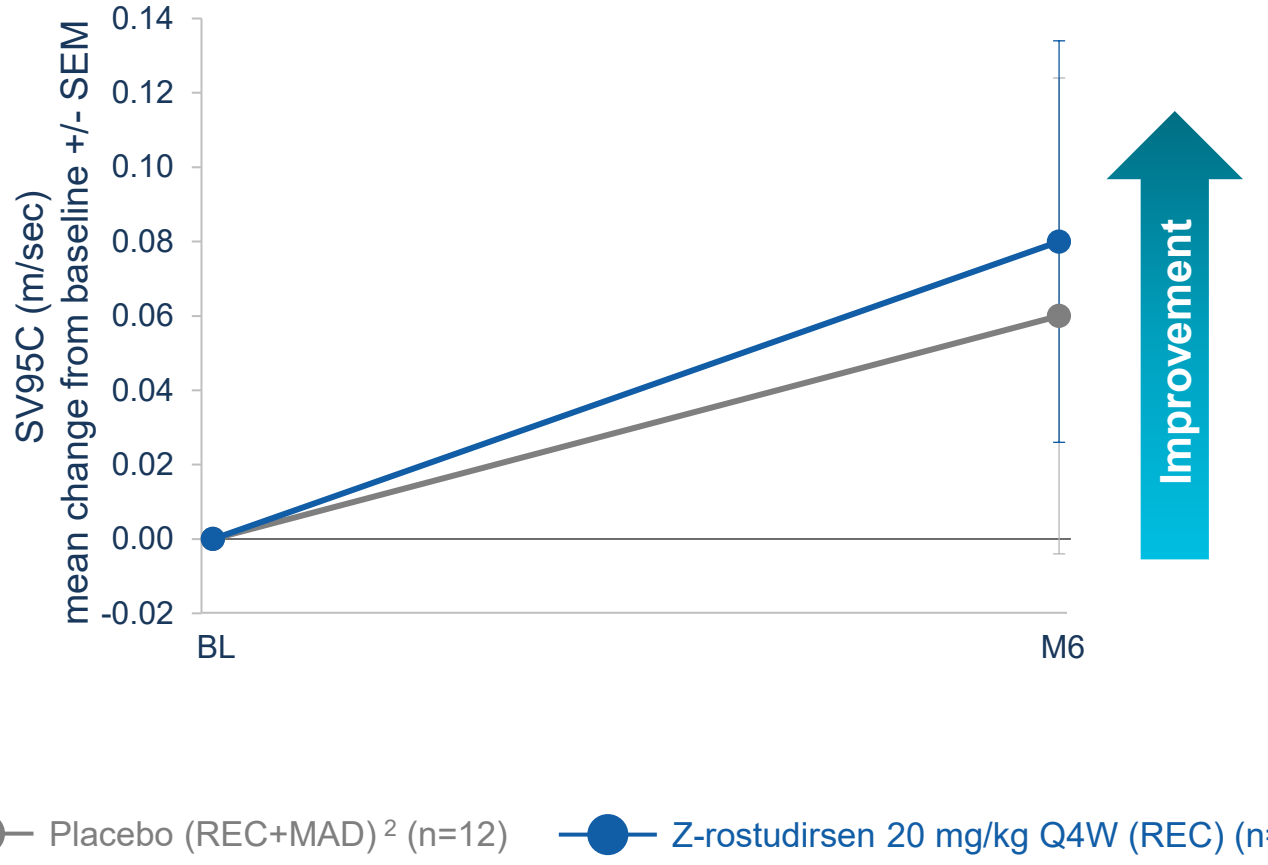
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1. Ambulant participants; missing values imputed. 2. Ayyar Gupta, et al. *PLoS One.* 2023;18:e0283669; MCID ≥2.3 points. 3. Post-hoc analysis; prespecified statistical analysis plan did not include formal hypothesis testing for any functional endpoint. BL, baseline; M, month; M6 = 169 days; MAD, multiple ascending dose MCID, minimal clinically important difference; Q4W, every 4 weeks; REC, registrational expansion cohort; SEM, standard error of the mean.

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Increase in SV95C at 6 months relative to baseline

Stride velocity 95th centile (SV95C)[1]



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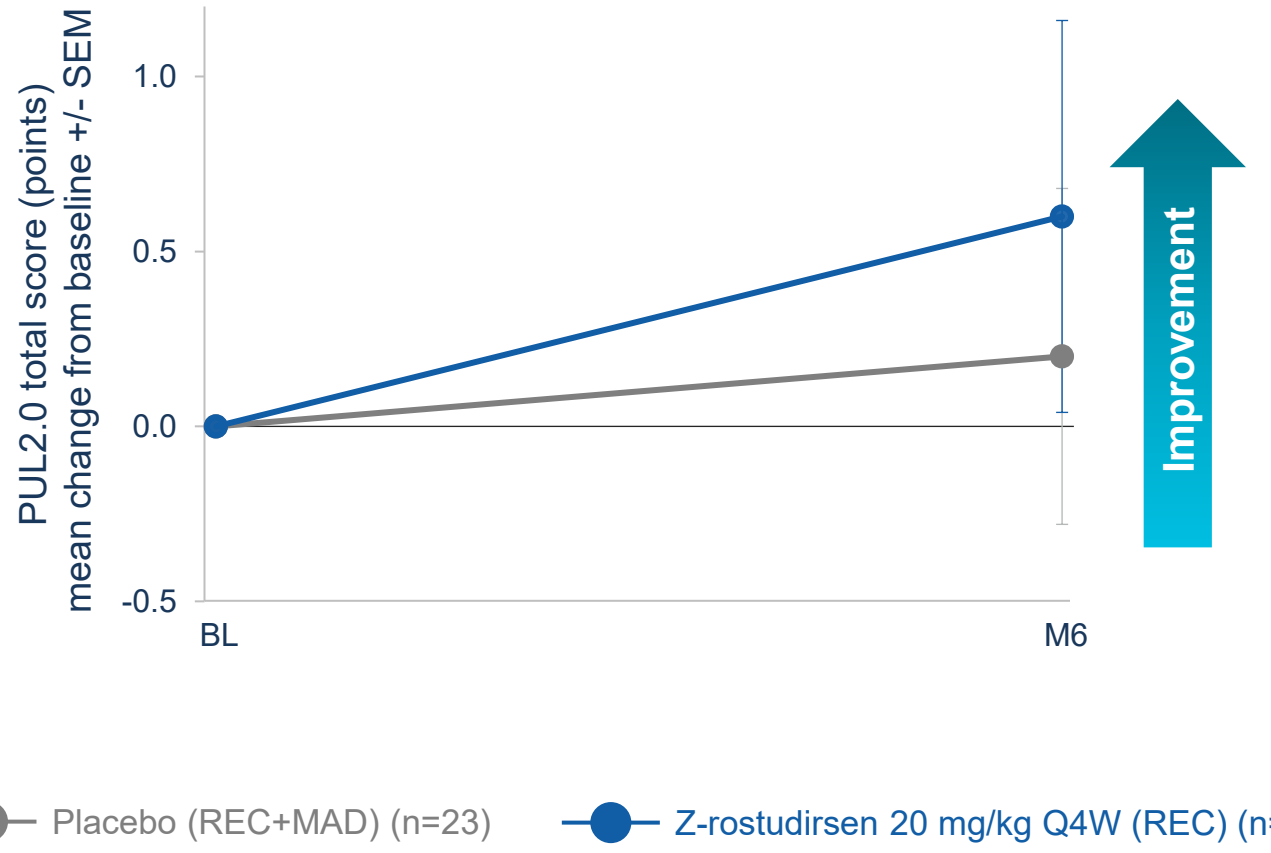
1. Ambulant participants; missing values imputed. 2. Placebo impacted by single participant with change from baseline of 0.46 m/sec at 6M; if this participant were excluded, mean change from baseline at 6M for placebo would be approximately 0.02 m/sec.

BL, baseline; M, month; m, meters; M6 = 169 days; MAD, multiple ascending dose; Q4W, every 4 week; REC, registrational expansion cohort; sec, seconds; SEM, standard error of the mean.

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PUL2.0 at 6 months in ambulant and non-ambulant participants relative to baseline and placebo

Performance upper limb v2.0 (PUL2.0)[1]



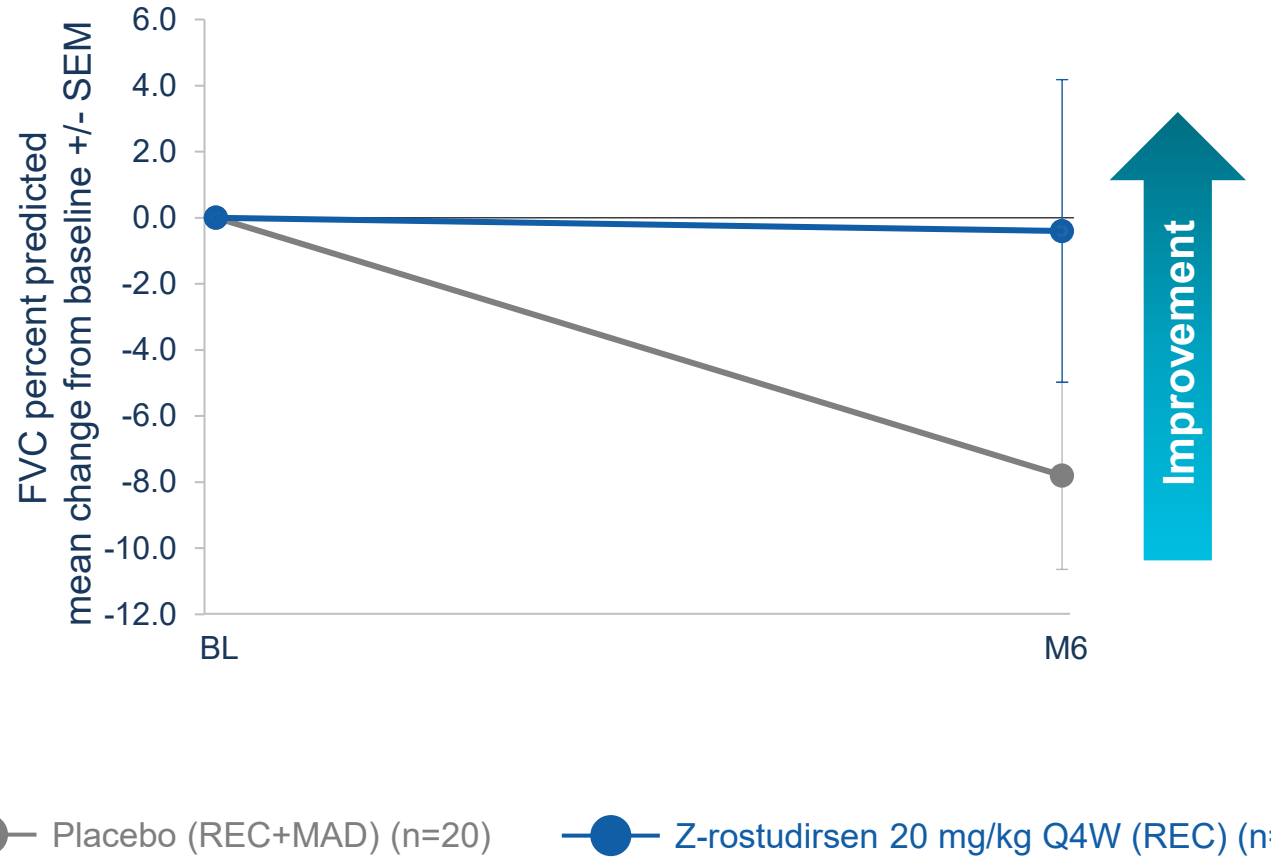
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1. Ambulant and non-ambulant participants; missing values imputed. BL, baseline; M, month; M6 = 169 days; MAD, multiple ascending dose; Q4W, every 4 weeks; REC, registrational expansion cohort; SEM, standard error of the mean.

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Preservation of lung function at 6 months

Forced vital capacity percent predicted (FVC%p)[1]



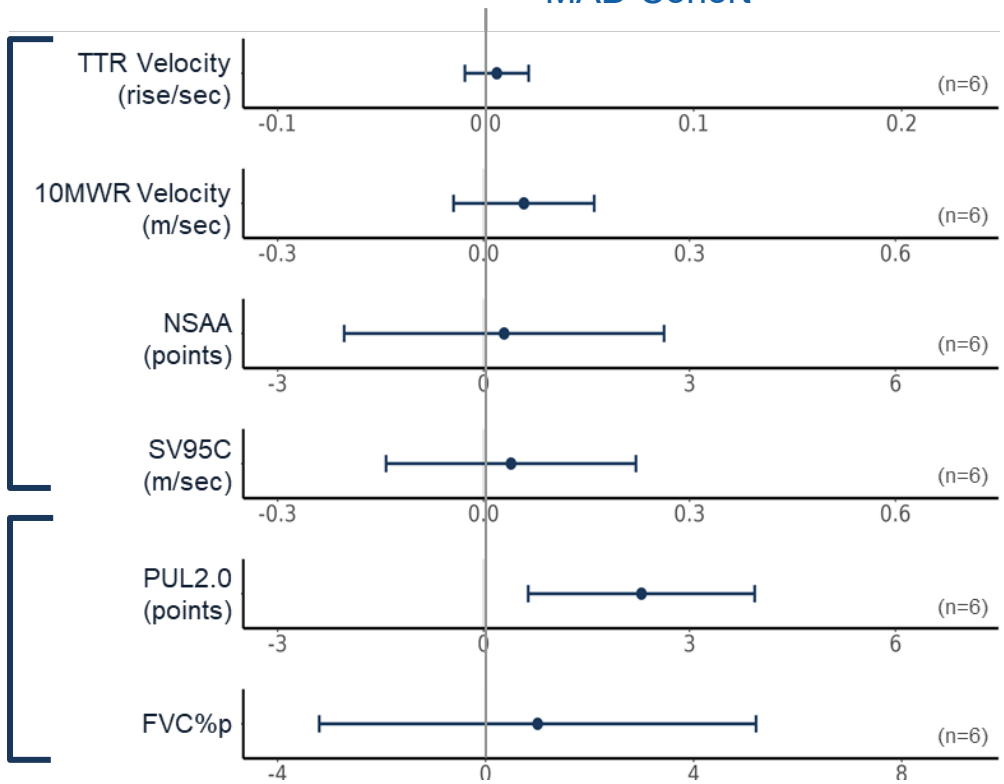
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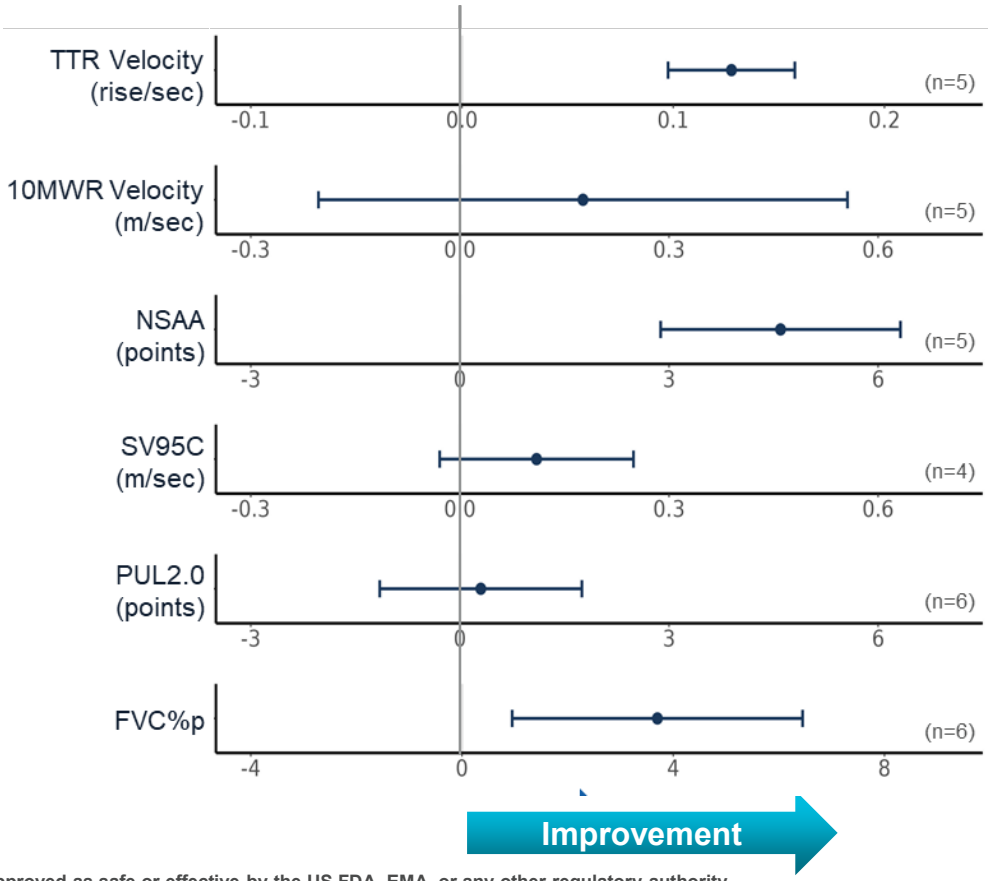
1. Ambulant and non-ambulant participants. BL, baseline; M, month; M6 = 169 days; MAD, multiple ascending dose; Q4W, every 4 weeks; REC, registrational expansion cohort; SEM, standard error of the mean.
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Functional improvement compared with baseline across all 6 measures up to 24 months in the OLE/LTE

Ambulatory participants

Ambulatory & non-ambulatory participants





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1. Mean change from baseline +/- SEM; TTR velocity, 10MWR velocity, NSAA, and SV95C analyzed from ambulant participants; PUL2.0 and FVC%p analyzed from ambulant and non-ambulant participants; Out-of-threshold and/or missing values imputed except for FVC%p.
2. Participants transitioned from 10 mg/kg Q4W to 20 mg/kg Q4W after 6M; all participants treated with 20 mg/kg Q4W for at least 12M in the 24M assessment. 10MWR, 10-meter walk/run; CFB, change from baseline; FVC%p, forced vital capacity percent predicted; LTE, long-term extension; m, meters; MAD, multiple ascending dose; NSAA, North Star Ambulatory Assessment; OLE, open-label extension; PUL2.0, performance upper limb v2.0; Q4W, every 4 weeks; sec, second; SEM, standard error of the mean; SV95C, stride velocity 95th centile; TTR, time to rise.
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Summary of DELIVER data

Endpoint	Patient population	Muscle system	6-month functional improvement vs. placebo*	24-month functional improvement vs. baseline†
TTR velocity	Ambulatory	Trunk & lower limbs	✓	✓
10MWR velocity	Ambulatory	Lower limbs	✓	✓
NSAA	Ambulatory	Upper limbs, trunk & lower limbs	✓	✓
SV95C	Ambulatory	Lower limbs	✓	✓
PUL2.0	Ambulatory & non-ambulatory	Upper limbs	✓	✓
FVC%p	Ambulatory & non-ambulatory	Diaphragm & trunk	✓	✓

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*Reflects results from 6-month placebo-controlled Registrational Expansion Cohort of DELIVER trial with 20 mg/kg Q4W z-rostudirsen. Check mark represents numerically higher value relative to placebo. †Reflects 24-month results from the long-term portions of the DELIVER trial for participants who transitioned from 10 mg/kg Q4W to 20 mg/kg Q4W after 6M; all participants treated with 20 mg/kg Q4W for at least 12M in the 24M assessment. Check mark represents numerically higher value relative to baseline.

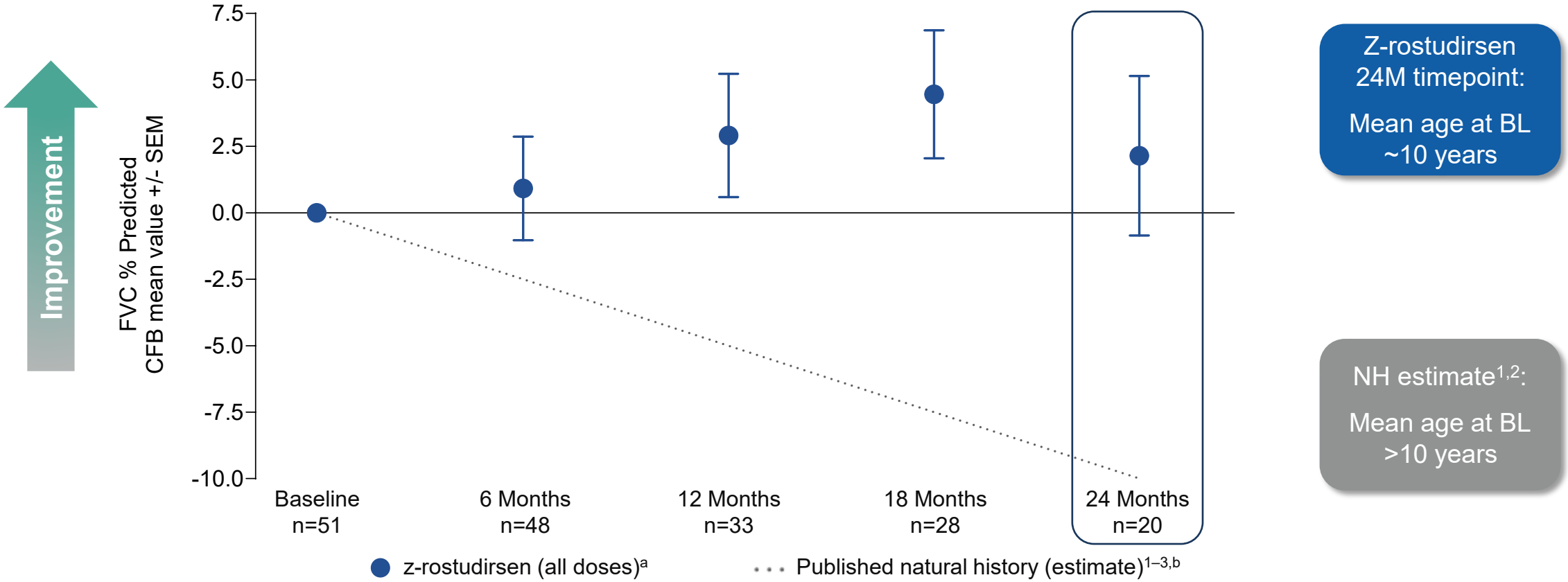
Considerations regarding dose escalation over time in a MAD study

Each timepoint represents a group of participants at varied dose levels and duration	
Dose levels	• Doses were escalated sequentially • Later timepoints included participants starting at low doses and who may have dose escalated through multiple doses
Dose duration	• Each timepoint reflects a group of participants with varying amounts of time on any specific dose

Considerations
• Most participants accrued substantial time on doses lower than 20 mg/kg Q4W prior to escalation • The observed long-term efficacy may not reflect the effect of continuously maintaining 20 mg/kg Q4W • There may be an underestimate of the true effect

MAD, multiple ascending dose; Q4W, every 4 weeks.

Long-term lung function with z-rostudirsen compared to published natural history data



Z-rostudirsen 24M timepoint:

Mean age at BL ~10 years

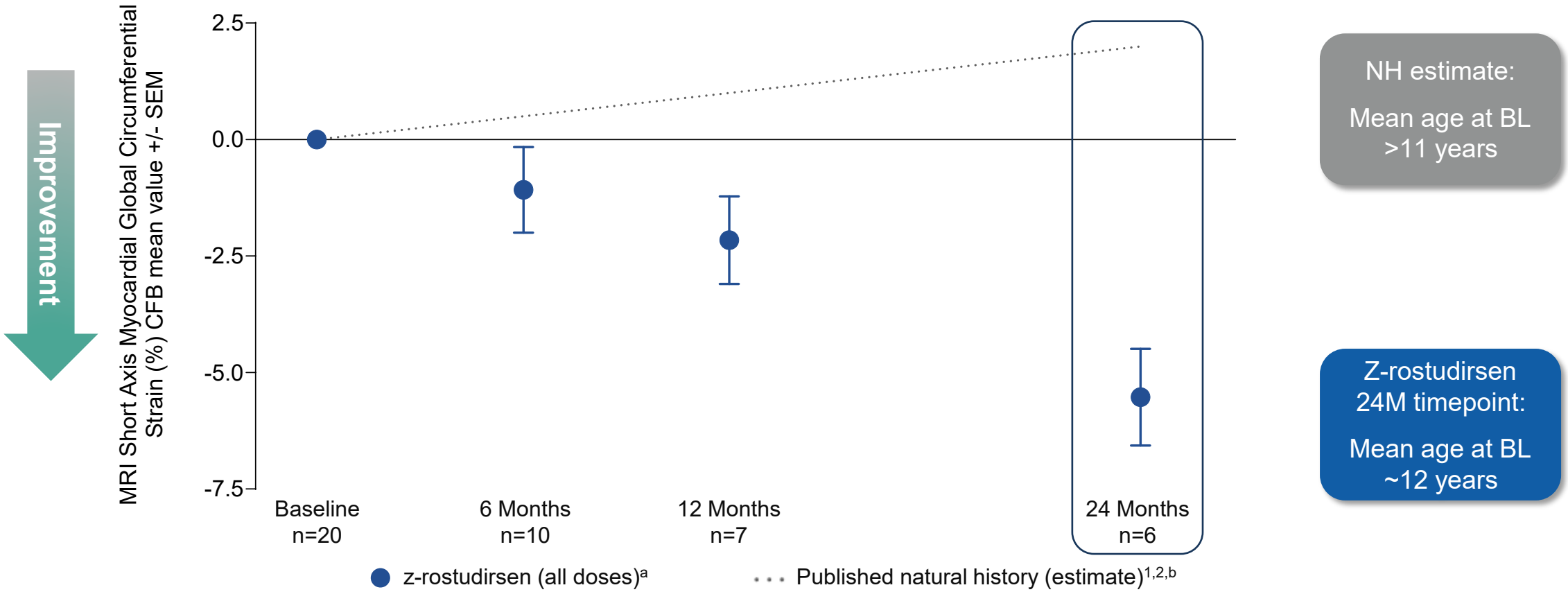
NH estimate[1,2]:

Mean age at BL >10 years

- The majority of participants at the 24M timepoint initiated treatment at the 0.7–2.8 mg/kg Q4W dose levels. One participant initiated treatment at 20 mg/kg Q4W
- All participants at the 24M timepoint received 20 mg/kg Q4W z-rostudirsen for ≥9 months

Results are reported as mean CFB through 24 months and presented with SEM.
a. Shown are all DELIVER participants (REC + MAD) randomized to z-rostudirsen treatment at baseline (any dose, including dose escalation[s] or de-escalation) and for whom FVC%p data were available. b. Based on 2-year published natural history data; assumes ~5%/year decline in FVC%p.
BL, baseline; CFB, change from baseline; FVC%p, forced vital capacity percent predicted; MAD, multiple ascending dose; NH, natural history; REC, registrational expansion cohort; SEM, standard error of the mean.
1. Mayer OH, et al. *Pediatr Pulmonol.* 2015;50:487–494; 2. McDonald CM, et al. *Neuromuscul Disord.* 2018;28:897–909; 3. Meier T, et al. *Neuromuscul Disord.* 2017;27:307–314.

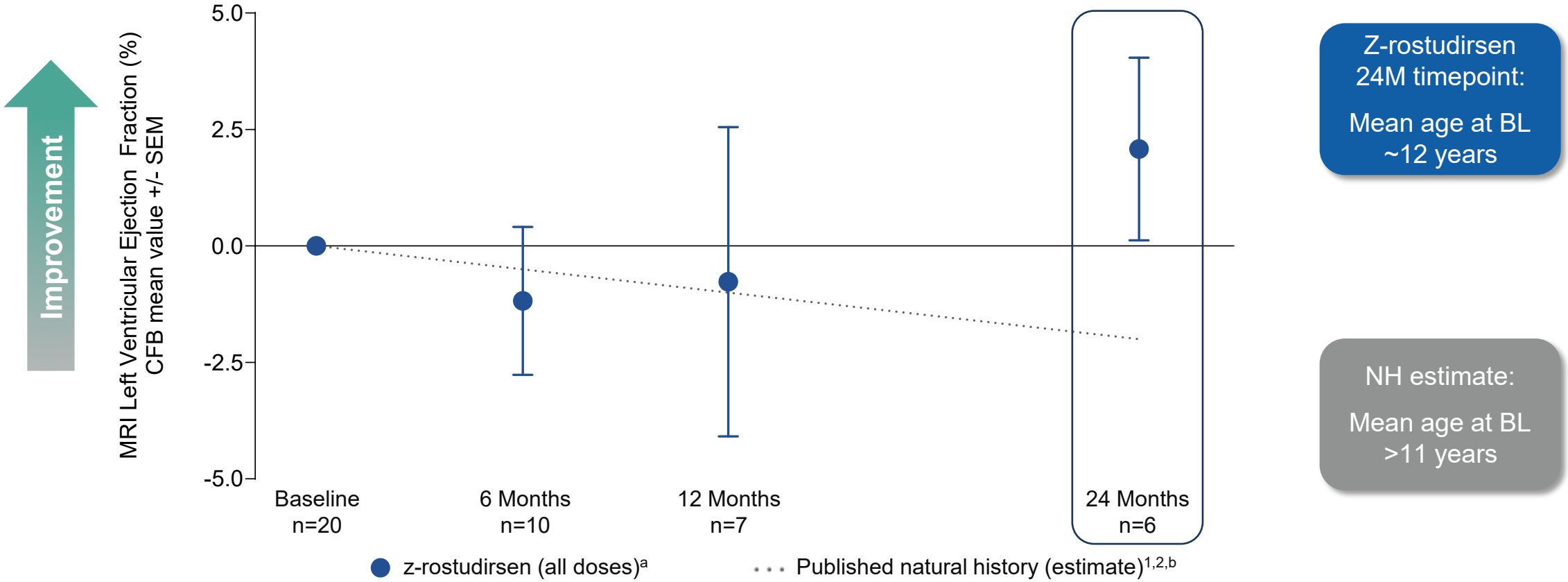
CMR-based circumferential strain in z-rostudiren-treated participants compared to published natural history data



- The majority of participants at the 24M timepoint initiated treatment at the 0.7–2.8 mg/kg Q4W dose levels. One participant initiated treatment at 20 mg/kg Q4W
- All participants at the 24M timepoint received 20 mg/kg Q4W z-rostudirsen for ≥6 months

Results are reported as mean CFB through 24 months and presented with SEM.
a. Shown are all DELIVER participants (REC + MAD) randomized to z-rostudirsen treatment at baseline (any dose, including dose escalation[s] or de-escalation) and for whom circumferential strain data were available. b. Based on 2-year published natural history data; assumes ~1%/year decline in strain.
BL, baseline; CFB, change from baseline; CMR, cardiac magnetic resonance; MAD, multiple ascending dose; MRI, magnetic resonance imaging; NH, natural history; REC, registrational expansion cohort; SEM, standard error of the mean.
1. Batra A, et al. *BMC Cardiovasc Disord.* 2022;22:260, *2.* Hagenbuch SC, et al. *Am J Cardiol.* 2010;105:1451–1455.

CMR-based left ventricular ejection fraction in z-rostudirsen-treated participants compared to published natural history data



Z-rostudirsen 24M timepoint:

Mean age at BL ~12 years

NH estimate:

Mean age at BL >11 years

- The majority of participants at the 24M timepoint initiated treatment at the 0.7–2.8 mg/kg Q4W dose levels. One participant initiated treatment at 20 mg/kg Q4W
- All participants at the 24M timepoint received 20 mg/kg Q4W z-rostudirsen for ≥6 months

Results are reported as mean CFB through 24 months and presented with SEM.
a. Shown are all DELIVER participants (REC + MAD) randomized to z-rostudirsen treatment at baseline (any dose, including dose escalation[s] or de-escalation) and for whom left ventricular ejection fraction data were available.
b. Based on 2-year published natural history data; assumes ~1%/year decline in LVEF.
BL, baseline; CFB, change from baseline; CMR, cardiac magnetic resonance; LVEF, left ventricular ejection fraction; MAD, multiple ascending dose; MRI, magnetic resonance imaging; NH, natural history; REC, registrational expansion cohort; SEM, standard error of the mean.
1. Batra A, et al. *BMC Cardiovasc Disord.* 2022;22:260, *2.* Hagenbuch SC, et al. *Am J Cardiol.* 2010;105:1451–1455.

Safety profile of z-rostudirsen 20 mg/kg Q4W

Summary of treatment-emergent adverse events (TEAEs)[1]

Study period	Placebo-controlled (PC) period (0 to 6M)		All study periods (0 to ≤36M)
Participants with ≥1 TEAE – n (%)	Placebo (MAD+REC) N=24[2]	Z-rostudirsen 20 mg/kg Q4W (MAD+REC) N=30[3]	Z-rostudirsen Pooled doses[4] (MAD+REC) N=85[5]
Any TEAE	22 (91.7)	29 (96.7)	80 (94.1)
Any related TEAE	3 (12.5)	10 (33.3)	41 (48.2)
Any serious TEAE	1 (4.2)	2 (6.7)	10 (11.8)
Any serious related TEAE	0	0	4 (4.7)
Any TEAE leading to withdrawal from study	0	0	0
Any TEAE leading to death	0	0	0

Most related TEAEs were mild or moderate

Potentially related serious TEAEs

- 2 participants at 20 mg/kg Q4W (registrational dose)
  - Pyrexia (fever) and malaise[6]
- 2 participants at 40 mg/kg Q4W
  - Acute kidney injury; thrombocytopenia[7]
  - Pancytopenia[8]

Most frequent related TEAEs ≥10%[9]

- Pyrexia (fever) (18%)
- Headache (13%)

Additional safety data at 20 mg/kg Q4W

- No participants have persistent related anemia[10] or thrombocytopenia

1,441 doses of z-rostudirsen administered to date representing 113 patient-years of follow-up (up to 36 months)[1]
1,062 doses of z-rostudirsen at 20 mg/kg dose level administered to date[1]

Zeleciment rostudirsen (z-rostudirsen, also known as DYNE-251) is an investigational medicine or otherwise in development and has not been approved as safe or effective by the US FDA, EMA, or any other regulatory authority.

1. Data as of August 19, 2025; all participants, placebo-controlled period, OLE, and LTE. 2. All placebo participants pooled from MAD and REC. 3. All participants randomized to z-rostudirsen 20 mg/kg Q4W in MAD and REC cohorts.
4. All doses of z-rostudirsen from MAD and REC at doses ranging from 0.7 mg/kg to 40 mg/kg every 4 or 8 weeks. 5. One participant randomized to placebo in REC not yet dosed with z-rostudirsen as of August 19, 2025. 6. One participant with same day onset of pyrexia and malaise in OLE and separate single event of pyrexia in LTE; one participant with single event of pyrexia in LTE; both participants fully recovered and have continued to receive z-rostudirsen without interruption. 7. Events had same day of onset in a single participant with a non-serious related TEAE of anemia in the context of fever, hemolysis, diarrhea, and positive blood in stool; together these events were consistent with hemolytic uremic syndrome with a possible infectious etiology. 8. Participant has a history of hemolytic anemia of unidentified etiology; presented with fever and tonsilitis; symptoms resolved without therapeutic intervention. 9. All cohorts combined; preferred terms reported. 10. No participants have persistent related anemia with Hgb levels <11.2 g/dL (threshold for anemia in children [ref: Powers JM. Approach to the child with anemia. UpToDate, Connor RF (Ed), Wolters Kluwer. Accessed December 2, 2025]).
Hgb, hemoglobin; LTE, long-term extension; M, months; MAD, multiple ascending dose; OLE, open-label extension; Q4W, every 4 weeks; REC, registrational expansion cohort; TEAE, treatment-emergent adverse event.
Dyne Therapeutics. Positive Topline Results from Phase 1/2 DELIVER Trial of Zeleciment Rostudirsen (DYNE-251) in DMD to Support Potential U.S. Accelerated Approval. Available from: https://investors.dyne-tx.com/static-files/2e54fea2-98b4-44a4-acaf-6277e0ce965e.

Summary

Investigational z-rostudirsen appears to show broad multisystem functional improvement in clinical measures across muscles of the upper and lower limb, the trunk, and cardiac and pulmonary muscles

The DELIVER REC met the primary endpoint, demonstrating a statistically significant increase in mean muscle content-adjusted dystrophin at 6 months compared with baseline

At 6 months, functional improvement was observed across multiple measures of muscle strength and function relative to baseline and placebo, including TTR velocity, 10MWR velocity, NSAA, and PUL2.0

Functional improvement was also seen over the longer term (up to 24 months) across these clinical measures, including measures of cardiac and pulmonary function

Z-rostudirsen demonstrated a favorable safety and tolerability profile based on data from 86 participants enrolled in DELIVER and followed for up to 36 months[1]

Data from the DELIVER trial support the potential of z-rostudirsen to address the unmet needs of individuals with DMD pathogenic variants amenable to exon 51 skipping

Zeleciment rostudirsen (z-rostudirsen, also known as DYNE-251) is an investigational medicine or otherwise in development and has not been approved as safe or effective by the US FDA, EMA, or any other regulatory authority.

1. Data as of August 19, 2025.

10MWR, 10-meter walk/run; DMD, Duchenne muscular dystrophy; NSAA, North Star Ambulatory Assessment; PUL, performance upper limb; REC, registrational expansion cohort; TTR, time to rise.



Even small, measurable changes – an extra few steps, an easier rise, a longer walk, retaining upper arm strength – can make a real difference for patients and families

THANK YOU



Dyne® THERAPEUTICS